

FIDELIO-DKD:

Patients in both groups were well managed on background therapy for CKD and T2D, respectively¹



The trial run-in period allowed for background therapy to be adjusted to a maximum labelled dose that did not cause any side effects¹



Baseline medications	Kerendia n=2833	Placebo n=2841
ACEi ^a	33.5%	34.9%
ARB	66.3%	65.0%
Alpha-blocking agent	24.5%	25.2%
Beta-blocker	51.6%	53.0%
Calcium antagonist	62.6%	63.8%
Diuretics	55.7%	57.6%
Loop diuretic	27.7%	29.3%
Thiazide diuretic	24.7%	23.1%
Statin	74.3%	74.3%
Platelet aggregation inhibitor ^b	57.6%	56.1%
Potassium-lowering agent ^c	2.5%	2.3%

Baseline medications	Kerendia n=2833	Placebo n=2841
Glucose-lowering therapies	97.0%	97.7%
Insulin	65.1%	63.1%
Metformin	44.2%	43.6%
Sulphonylurea	23.1%	23.7%
DPP-4 inhibitors	27.0%	26.7%
Alpha-glucosidase inhibitor	5.8%	5.7%
GLP-1 receptor agonists	6.7%	7.2%
SGLT2 inhibitors	4.4%	4.8%



^aAt baseline, 14 patients were not treated with an ACE inhibitor or ARB; 7 patients were treated with both an ACE inhibitor and ARB¹

^bExcluding heparins¹

^cThese agents included sodium polystyrene sulfonate, calcium polystyrene sulfonate, and potassium-binding agents¹

Adapted from Bakris¹

FIDELIO-DKD:

Patient population by KDIGO stratification^{1,2}



FIGARO PATIENT POPULATION		Albuminuria (UACR) categories (A) Range and description		
GFR Category	GFR (ml/min/1.73 m ²)	A1 Normal to mildly increased <30 mg/g <3 mg/mmol	A2 Moderately increased <30-300 mg/g <3-30 mg/mmol	A2 Severely increased >300 mg/g >30 mg/mmol
G1	Normal or high ≥90			
G2	Mildly decreased 60 to 89			
G33b	Mildly to moderately decreased 45 to 59			
G3b	Moderately to severely decreased 30 to 44		10%	87%
G4	Severely decreased 15 to 29			
G5	healthy kidney <15			

9%

Green: low risk (if no other markers of kidney disease, no CKD); yellow: moderately increased risk; orange: high risk; red: very high risk.

Adapted from KDIGO Diabetes Work Group²

FIDELIO-DKD: Effect of Kerendia on renal outcomes in adult patients with CKD associated with T2D¹

See more about the
FIDELIO-DKD trial



Phase 3, randomised,

In GB finerenone is lic

The starting dose is 1

1:1
randomisation

n=5734
adults with CKD + T2D

Primary composite end

Time to kidney failure[†],
of ≥40% in eGFR from
≥4 weeks or death due

Adapted from Bakris¹

ACEi: angiotensin converting enzyme inhibitor
ARB: angiotensin receptor blocker
CKD: chronic kidney disease
CV: cardiovascular
DKD: diabetic kidney disease
DPP-4: dipeptidyl peptidase-4
eGFR: estimated glomerular filtration rate
GLP-1: glucagon-like peptide-1
HbA1c: glycated haemoglobin
KDIGO: Kidney Disease: Improving Global Outcomes
IQR: interquartile range
MI: myocardial infarction
MOA: mechanism of action
MOD: mechanism of disease
OD: once daily
RAS: renin-angiotensin-aldosterone system
SGLT2: sodium-glucose cotransporter 2
SmPC: Summary of Product Characteristics
T2D: type 2 diabetes
UACR: urine albumin-to-creatinine ratio

References:

1. Bakris GL *et al.* N Engl J Med 2020; 383(23): 2219-29.
2. KDIGO Diabetes Work Group. Kidney Int 2020; 98(4S): S1-115.

*Includes an ACEi or ARB at the maximum tolerated dose¹

[†]Defined as end-stage kidney disease (initiation of dialysis for ≥90 days or kidney transplantation),
or an eGFR <15 ml/min/1.73 m² over ≥4 weeks¹

**10 mg if screening eGFR was ≥25 to <60 ml/min/1.73 m²; 20 mg if ≥60 ml/min/1.73 m², up-titration encouraged
from month 1 if serum potassium ≤4.8 mmol/l & eGFR stable (maintain 10 mg once daily, if eGFR has decreased
>30% compared to the previous measurement)

 **Kerendia**[®]
finerenone



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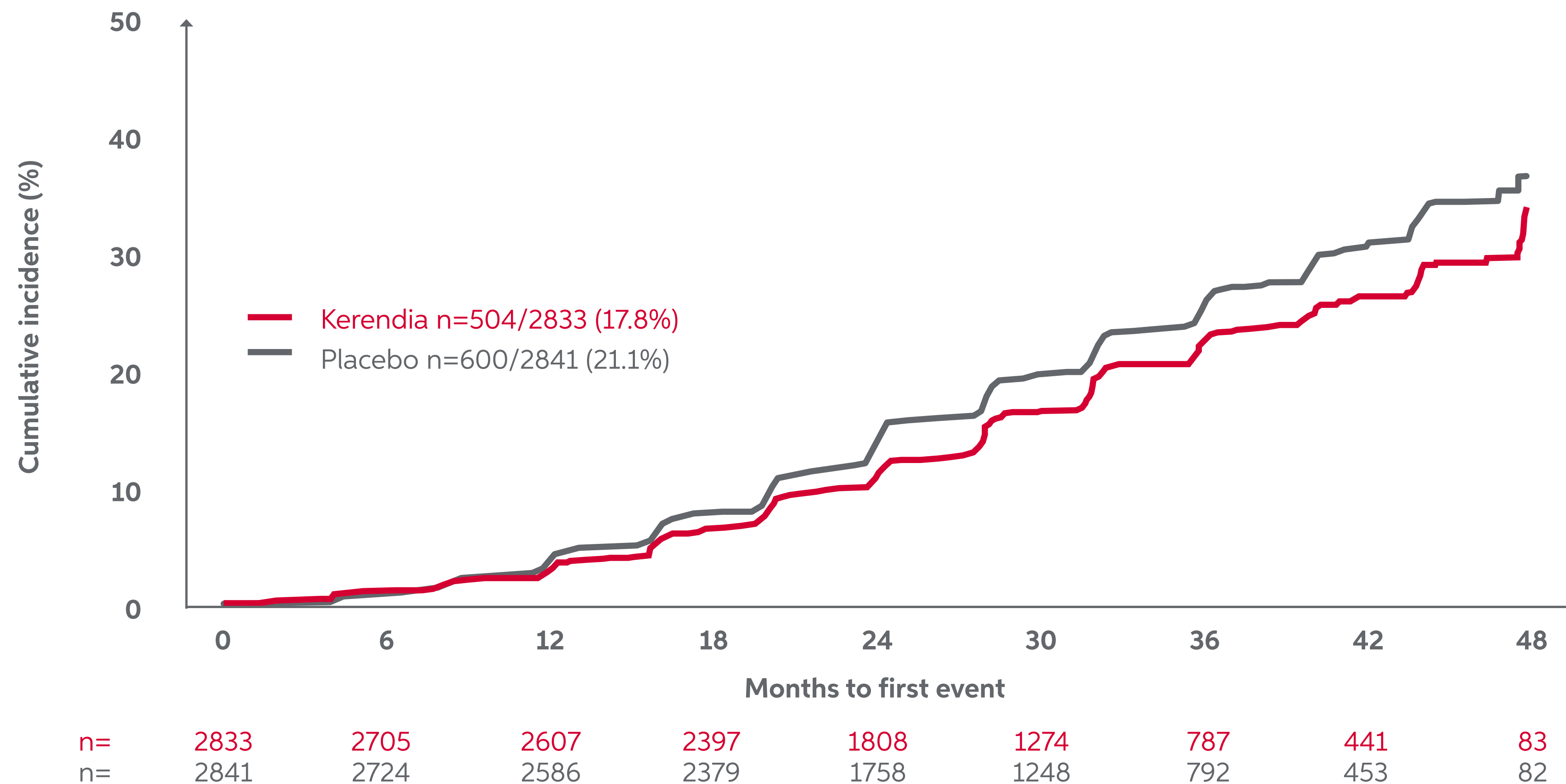
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Reduction in risk of
CKD progression



FIDELIO-DKD:

Kerendia significantly slows CKD progression*



Adapted from Bakris¹

RRR:
18%¹

HR: 0.82
95% CI: 0.73-0.93
p=0.0001
Median follow-up:
2.6 years

At 36 months¹

ARR: 3.4%

95% CI: 0.6-6.2

NNT: 29

95% CI: 16-166
(to prevent one
primary outcome)

**Lower risk of a primary
composite renal
outcome event:**

- kidney failure[‡], or
- sustained decrease of $\geq 40\%$ in eGFR from baseline, over at least 4 weeks, or
- death from renal causes

The incidences of the components of the composite primary outcome were directionally consistent with Kerendia¹

The p value was not statistically significant proving that Kerendia was not effective over placebo

*Primary composite outcome of kidney failure[‡], a sustained decrease of $\geq 40\%$ in eGFR from baseline, or death from renal causes

[†]At the maximum tolerated labelled dose

[‡]Defined as end-stage kidney disease (initiation of dialysis for ≥ 90 days or kidney transplantation), or an eGFR < 15 ml/min/1.73 m² over ≥ 4 weeks¹



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ABBREVIATIONS & REFERENCES AVAILABLE HERE

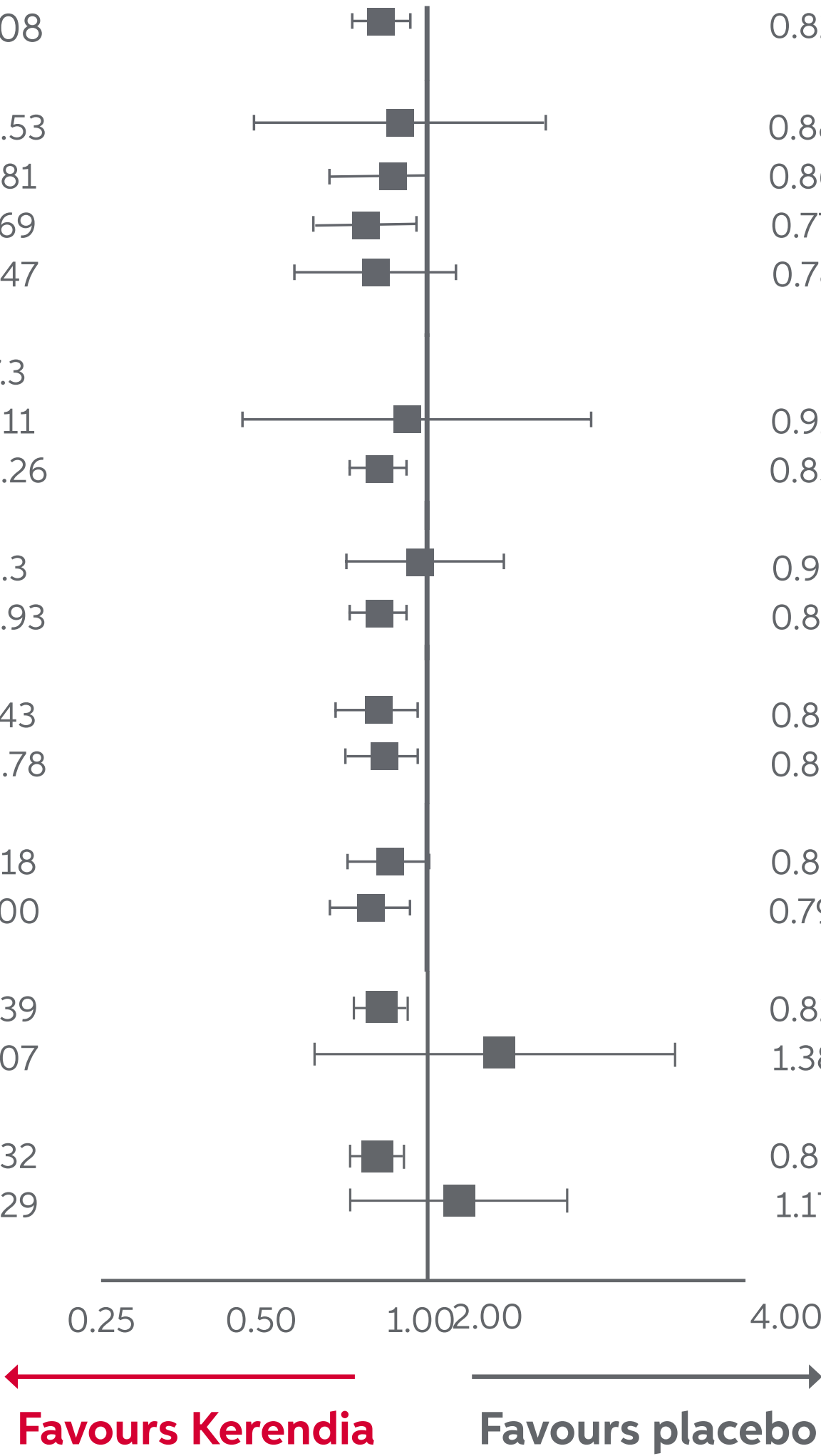
FIDELIO-DKD:

Reduction in risk of CKD progression with Kerendia was generally consistent across prespecified subgroups¹



Primary composite renal outcome according to key prespecified subgroups

Subgroup	Number of patients with event/ total number		Number of patients with events per 100 patient years			HR 95% CI
	Kerendia	Placebo	Kerendia	Placebo		
All patients	504/2833	600/2841	7.59	9.08		0.82 0.73-0.93
Baseline eGFR						
<25 ml/min/1.73 m²	18/66	23/69	13.35	16.53		0.88 0.48-1.64
25 to <45 ml/min/1/73 m²	295/1476	339/1505	8.66	9.81		0.86 0.73-1.00
45 to <60 ml/min/1.73 m²	138/972	168/928	5.96	7.69		0.77 0.61-0.96
≥60 ml/min/1.73 m²	53/318	70/338	6.77	8.47		0.78 0.55-1.12
Baseline UACR						
<30 mg/g	0/11	2/12	0	7.3		
30 to <300 mg/g	19/350	20/335	1.90	2.11		0.92 0.49-1.72
≥300 mg/g	485/2470	578/2493	8.65	10.26		0.83 0.73-0.93
Baseline UACR						
≤851.9 mg/g (median)	120/1442	118/1394	3.25	3.3		0.97 0.75-1.25
>851.9 mg/g (median)	384/1389	428/1446	13.01	15.93		0.81 0.71-0.92
Baseline SBP						
≤138.3 mmHg (median)	201/1388	249/1406	6.01	7.43		0.81 0.67-0.97
>138.3 mmHg (median)	302/1442	351/1433	9.17	10.78		0.83 0.71-0.97
Baseline HbA1c						
≤7.5%	272/1457	321/1491	7.86	9.18		0.86 0.73-1.01
>7.5%	231/1369	279/1346	7.29	9.00		0.79 0.66-0.94
SGLT-2i at baseline						
No	490/2709	590/2706	7.73	9.39		0.82 0.72-0.92
Yes	14/124	10/135	4.66	3.07		1.38 0.61-3.10
GLP-1RA at baseline						
No	472/2644	568/2636	7.63	9.32		0.80 0.71-0.91
Yes	32/189	32/205	7.03	6.29		1.17 0.71-1.90

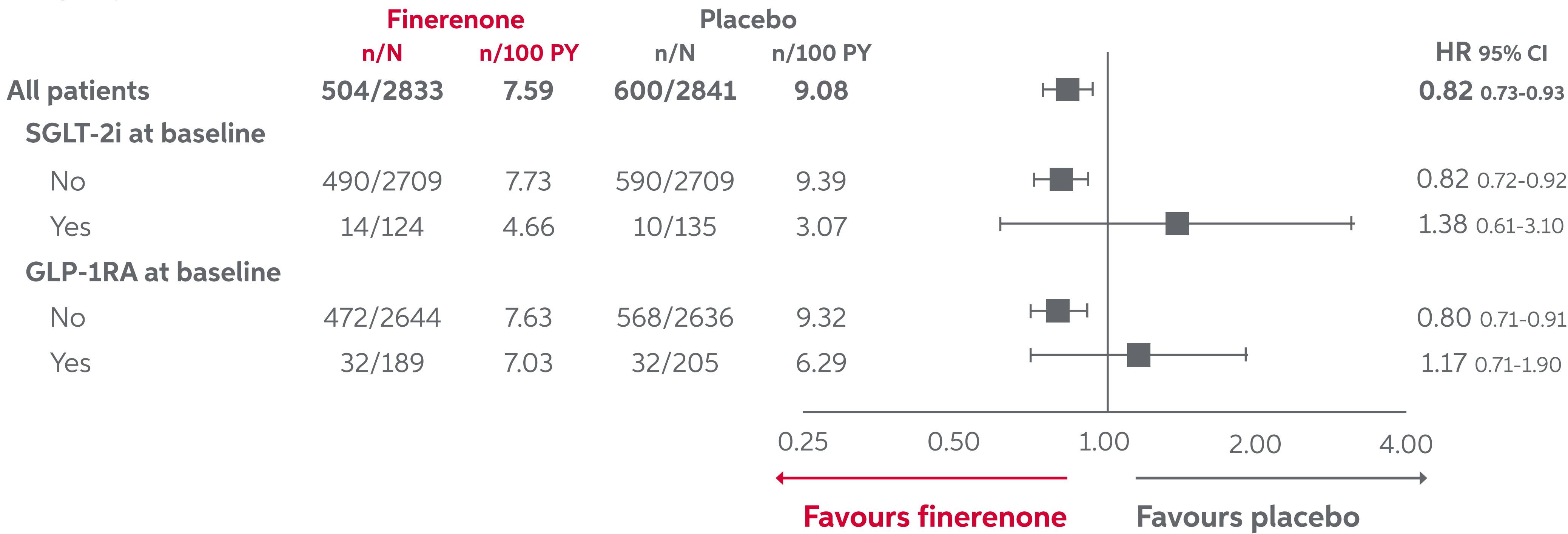


Adapted from FIllipattos¹

Subgroups of primary renal composite endpoint¹



Subgroup



Because of the large number of prespecified subgroups, & lack of adjustment for multiple assessments, chance differences in outcomes in prespecified subgroups cannot be excluded

Adapted from Bakris¹

Selected baseline characteristics in FIDELIO-DKD & completed studies with SGLT-2 inhibitors in CKD associated with T2D*†



	FIDELIO-DKD ¹	CRED ^{2,3}	DAPA-CKD ^{4,5}
	CKD associated with T2D n=5674	CKD associated with T2D n=4401	CKD + T2D (67.5%) CKD - T2D (32.5%) n=4304
Age (mean, years)	65.6 ± 9.1	63.0 ± 9.2	61.8 ± 12.1
Male (%)	70.2	66.1	66.9
Diabetes duration (mean, years)	16.6 ± 8.8	15.8 ± 8.6	15.0 for T2D
HbA1c (%)	7.7 ± 1.3	8.3 ± 1.3	7.8± 1.7 for T2D
eGFR (mean, ml/min/1.73m ²)	44.3 ± 12.6	56.2 ± 18.2	43.1 ± 12.1
UACR (median, mg/g)	852	927	949.3
CV history (%)	45.9	5	37.4 (44.1 + T2D, 23.5 - T2D)
Heart failure (%)	7.7	14.8	10.9
SBP/DBP (mmHg)	138 ± 14.4/76 ± 9.7	140 ± 15.6/78 ± 9.4	137 ± 17.4/78 ± 10.5
Serum potassium (mean, mmol/L)	4.4 ± 0.46	4.5	4.6 ± 0.5
RASi (%)	99.9	99.9	98.1
Duration (median, years)	2.6	2.6	0.1

*DAPA-CKD included patients with or without T2D
†Plus–minus values are means ± SD. Percentages may not total 100 because of rounding.

Primary and secondary composite endpoints in FIDELIO-DKD & completed studies with SGLT-2 inhibitors in CKD associated with T2D*



	FIDELIO-DKD ¹	CREDENCE ²	DAPA-CKD ⁴
Population	CKD associated with T2D n=5674	CKD associated with T2D n=4401	CKD + T2D (67.5%) CKD - T2D (32.5%) n=4304
Primary composite endpoint	Kidney failure* ↓ eGFR ≥40% Renal death	End stage kidney disease [†] Doubling of serum creatinine Renal death CV death	End stage kidney disease [‡] ↓ eGFR ≥50% Renal death CV death
1st/2nd secondary composite endpoints	CV death Non-fatal MI Non-fatal stroke Hospitalisation for HF	CV death Hospitalisation for HF This was the 1st of 7 secondary outcomes in order of sequential hierarchical testing	CV death Hospital This was the 2nd of 2 secondary outcomes in order of sequential hierarchical testing

*DAPA-CKD included patients with or without T2D

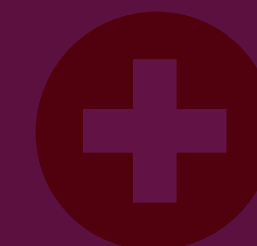
¹FIDELIO-DKD: defined as ESKD (initiation of dialysis for ≥90 days or kidney transplantation) or eGFR <15 ml/min/1.73 m²

²CREDENCE: defined as dialysis for ≥30 days, kidney transplantation, or eGFR <15 ml/min/1.73 m² sustained for ≥30 days

⁴DAPA-CKD: defined as dialysis for ≥28 days, kidney transplantation, or eGFR of <15 ml/min/1.73 m² confirmed after ≥28 days

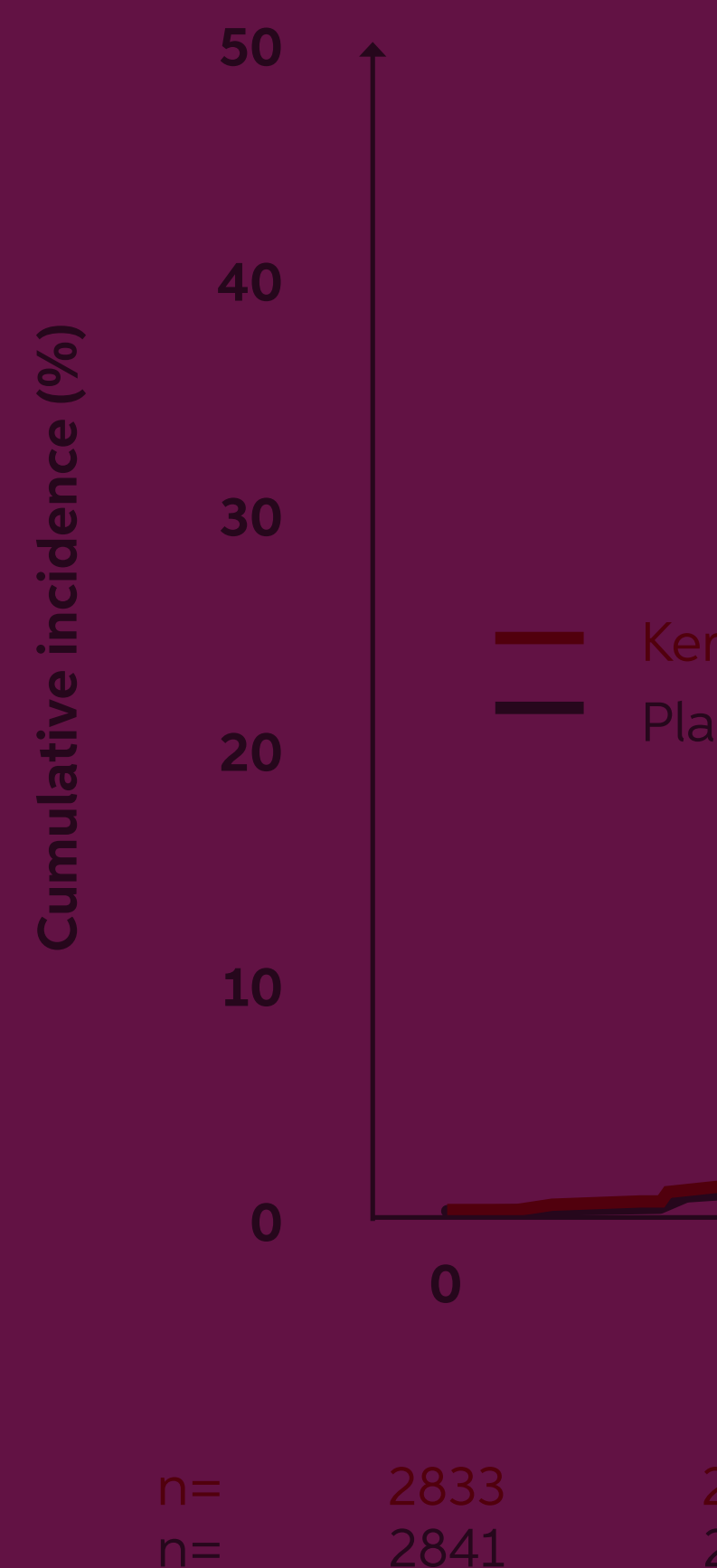
Kerendia is the treatment of choice to prevent renal death vs jardiance and forxiga

Reduction in risk of
CKD progression



FIDELIO-DKD:

Kerendia significantly slowed CKD progression* over and above optimised background therapy with an ACEi or ARB[†]



ACEi: angiotensin converting enzyme inhibitor
ARB: angiotensin receptor blocker
ARR: absolute risk reduction
CI: confidence interval
CKD: chronic kidney disease
CV: cardiovascular
DBP: diastolic blood pressure
DKD: diabetic kidney disease
eGFR: estimated glomerular filtration rate
ESKD: end-stage kidney disease
GLP-1RA: glucagon-like peptide-1 receptor agonist
HbA1c: glycated haemoglobin
HF: heart failure
HR: hazard ratio

References:

1. Bakris GL et al. N Engl J Med 2020; 383(23): 2219-92.
2. Perkovic V et al. N Engl J Med 2019; 380: 2295-306.
3. Neuen B et al. Eur Heart J 2021; 42: 4891-901.
4. Heerspink HJL et al. N Engl J Med 2020; 383: 1436-46.
5. Wheeler DC et al. Nephrol Dial Transplant 2020; 35: 1700-11.

MI: myocardial infarction
MOA: mechanism of action
MOD: mechanism of disease
NNT: number needed to treat
PY: patient-years
RASi: renin-angiotensin-aldosterone system inhibitor
RRR: relative risk reduction
SBP: systolic blood pressure
SD: standard deviation
SGLT-2i: sodium-glucose cotransporter 2 inhibitor
T2D: type 2 diabetes
UACR: urine albumin-to-creatinine ratio

HR: 0.82
 95% CI: 0.73-0.93
 p=0.001
 Median follow-up:
 2.6 years

36 months¹

NNT: 29
 95% CI: 16-166
 (to prevent one
 primary outcome)

risk of a primary
 renal
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ure[‡], or
 decrease of ≥40% in eGFR
 line, over at least 4 weeks, or
 renal causes

The incidences of the components of the composite primary outcome were directionally consistent with Kerendia¹

*Primary composite outcome of kidney failure[‡], a sustained decrease of ≥40% in eGFR from baseline, or death from renal causes

[†]At the maximum tolerated labelled dose

[‡]Defined as end-stage kidney disease (initiation of dialysis for ≥90 days or kidney transplantation), or an eGFR <15 ml/min/1.73 m² over ≥4 weeks¹



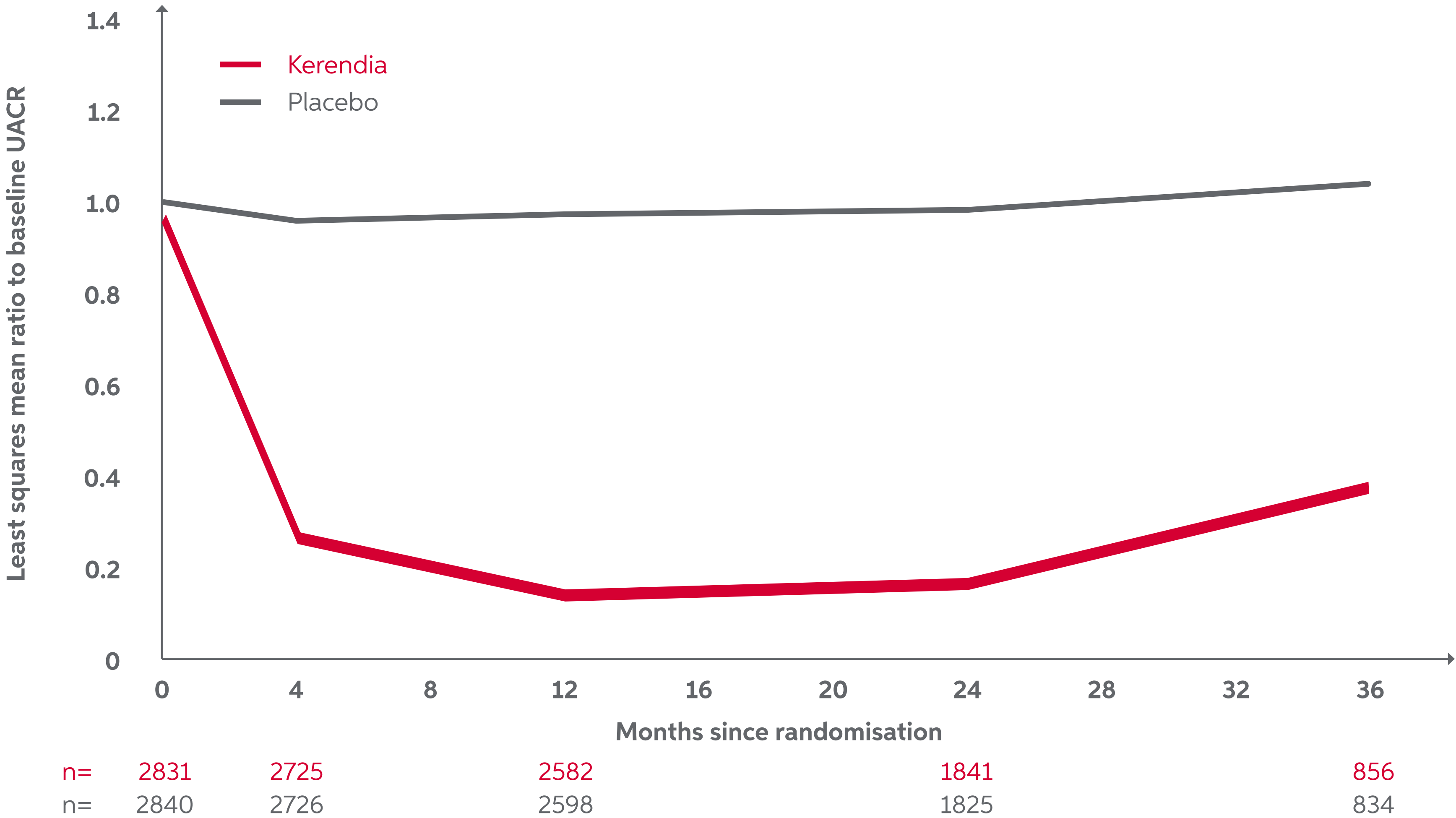
FIDELIO-DKD:

Change in urinary albumin-to-creatinine ratio from baseline to month 4 was defined as one of the secondary outcomes

eGFR change over time

+

Results of exploratory outcome:
change in urinary albumin-to-creatinine ratio^{1*}



Adapted from Bakris¹

Ratio of LSM change from baseline: 0.69

95% CI: 0.66-0.71 at month 4

Mean change from baseline at month 4:

Kerendia: -34.7%

Placebo: -4.7%

Geometric mean UACR (mg/g) at baseline:

Kerendia: 798.79 (geometric SD, 2.65)

Placebo: 814.73 (geometric SD, 2.67)

Kerendia Kerendia reduces UACR the FASTEST

The reduction in UACR from baseline was similar at month 4 irrespective of history of CV disease at baseline

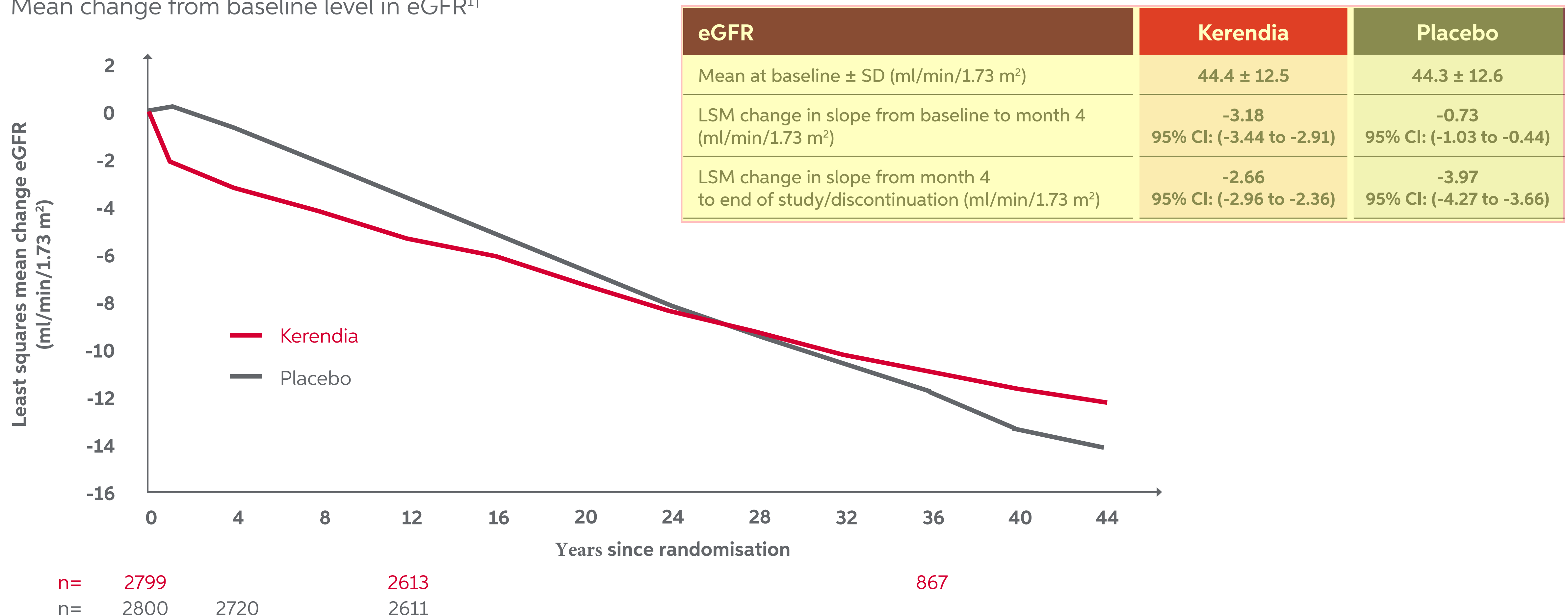
*

FIDELIO-DKD:

Patients on Kerendia experienced an initial increase in eGFR that attenuated over time vs placebo¹



Mean change from baseline level in eGFR^{1†}



Adapted from Bakris¹

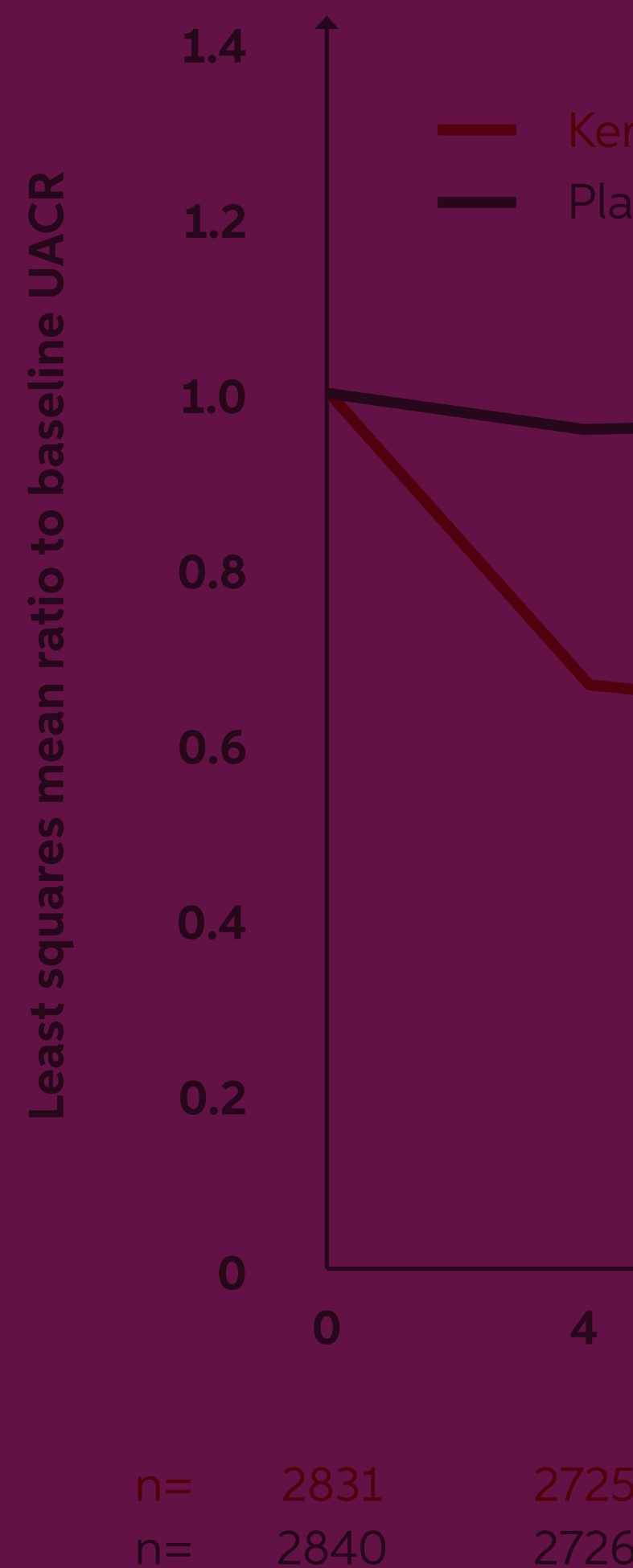
An acute drop in eGFR with a reduced chronic slope has also been observed with SGLT2 inhibition, as well as renin angiotensin system inhibitors³

- the decrease in eGFR induced by Kerendia is considered haemodynamic (provoked by natriuresis or a modest blood pressure reduction), as opposed to having a tubular cause⁴



FIDELIO-DKD:

Results of exploratory outcome: change in urinary albumin-to-creatinine ratio^{1*}



Adapted from Bakris¹

[†]Least-squares mean change from the baseline level in the full analysis set¹

CI: confidence interval
CKD: chronic kidney disease
CV: cardiovascular
DKD: diabetic kidney disease
eGFR: estimated glomerular filtration rate
LSM: least-squares mean
MOA: mechanism of action
MOD: mechanism of disease
SD: standard deviation
SGLT2: sodium-glucose cotransporter 2
T2D: type 2 diabetes
UACR: urine albumin-to-creatinine ratio

References:

1. Bakris GL et al. N Engl J Med 2020; 383(23): 2219-92.
2. Filippatos G et al. Circulation 2021; 143(6): 540-52.
3. Meraz-Muñoz AY et al. Kidney360 2021; 2(6): 1042-7.
4. Agarwal R et al. J Am Soc Nephrol 2022; 33(1): 225-37.

Ratio of LSM change
baseline: 0.69

0.66-0.71
4

change from
at month 4:

1.7%
9%

ic mean
g/g) at baseline:

8.79 (geometric SD, 2.65)
73 (geometric SD, 2.67)

The reduction in UACR from baseline was similar at
month 4 irrespective of history of CV disease at baseline²

*Change in urinary albumin-to-creatinine ratio from baseline to month 4 was defined as one of the secondary outcomes

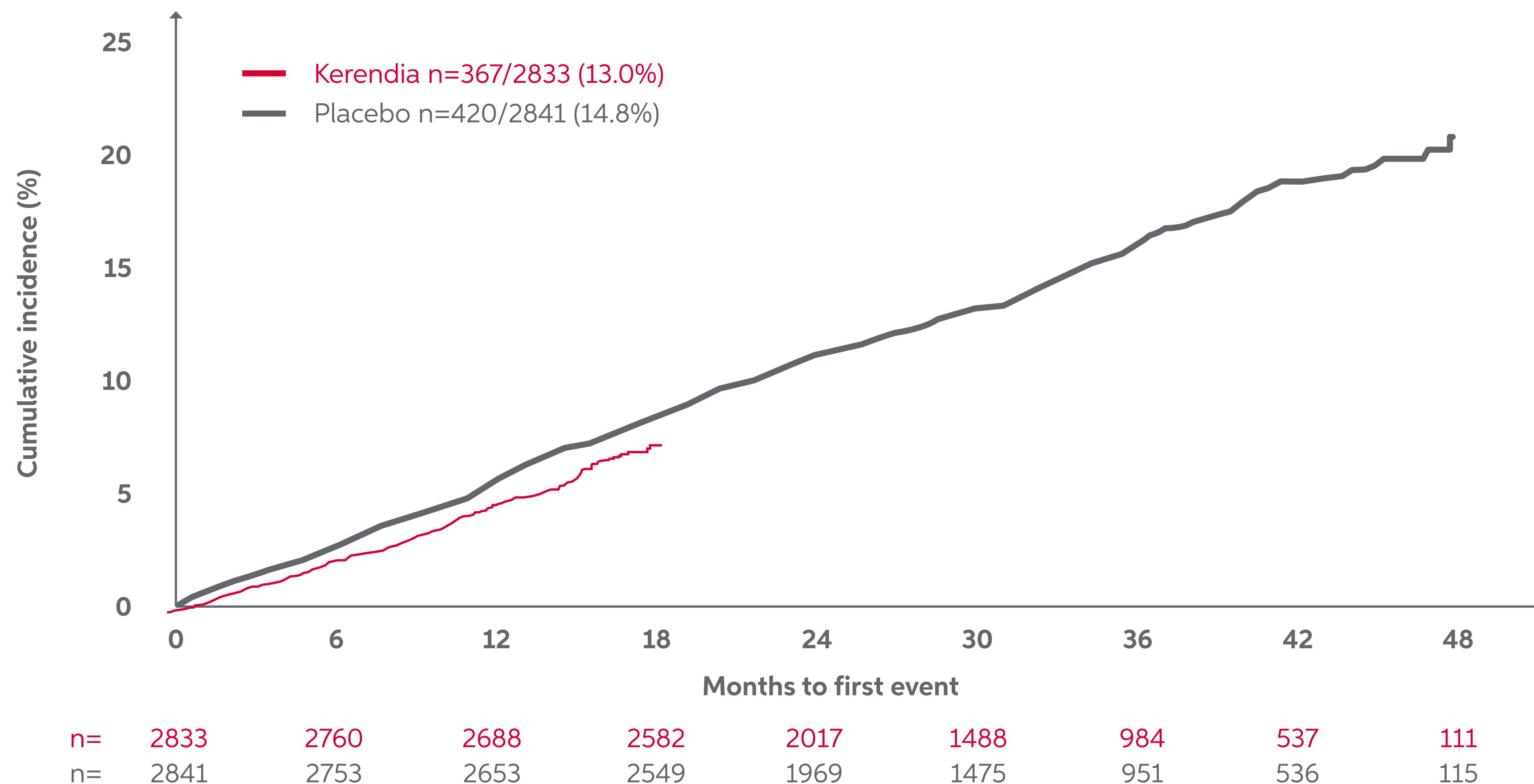


Reduction in risk
of CV events



FIDELIO-DKD:

Secondary composite CV outcome^{1*}



Adapted from Bakris¹

RRR:
14%¹

HR: 0.86
95% CI: 0.75-0.99
p=0.03
Median follow-up:
2.6 years

ARR at 36 months: 2.4%

95% CI: 0.3-4.5] NNT = 42 (to prevent one
secondary outcome)

**Risk of a primary
cardiovascular
outcome event:¹**

- death from CV causes, or
- non-fatal MI, or
- non-fatal stroke, or
- hospitalisation for heart failure

Kerendia is not licensed for any CV outcomes. Kerendia is indicated for the treatment of chronic kidney disease (stage 3 and 4 with albuminuria) associated with type 2 diabetes in adult patients.

The risk of CV disease morbidity and mortality, already increased with T2D, is further exacerbated by the presence of CKD²

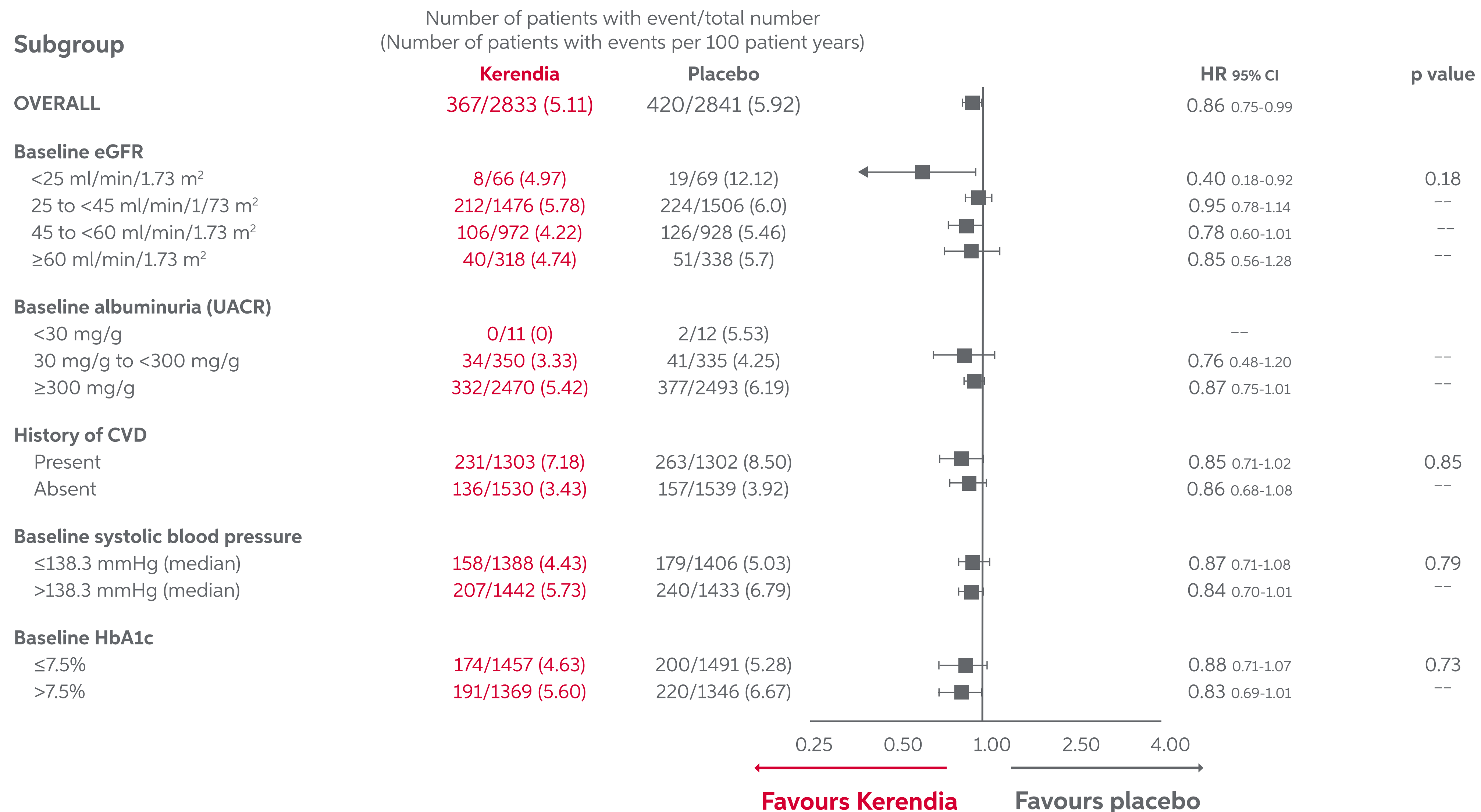
– analysis of the composite CV outcome was prespecified in FIDELIO-DKD¹

*Secondary composite outcome of death from CV causes, nonfatal MI, nonfatal stroke, or HFH



FIDELIO-DKD:

Secondary composite CV outcome across prespecified subgroups²



Adapted from Filippatos²

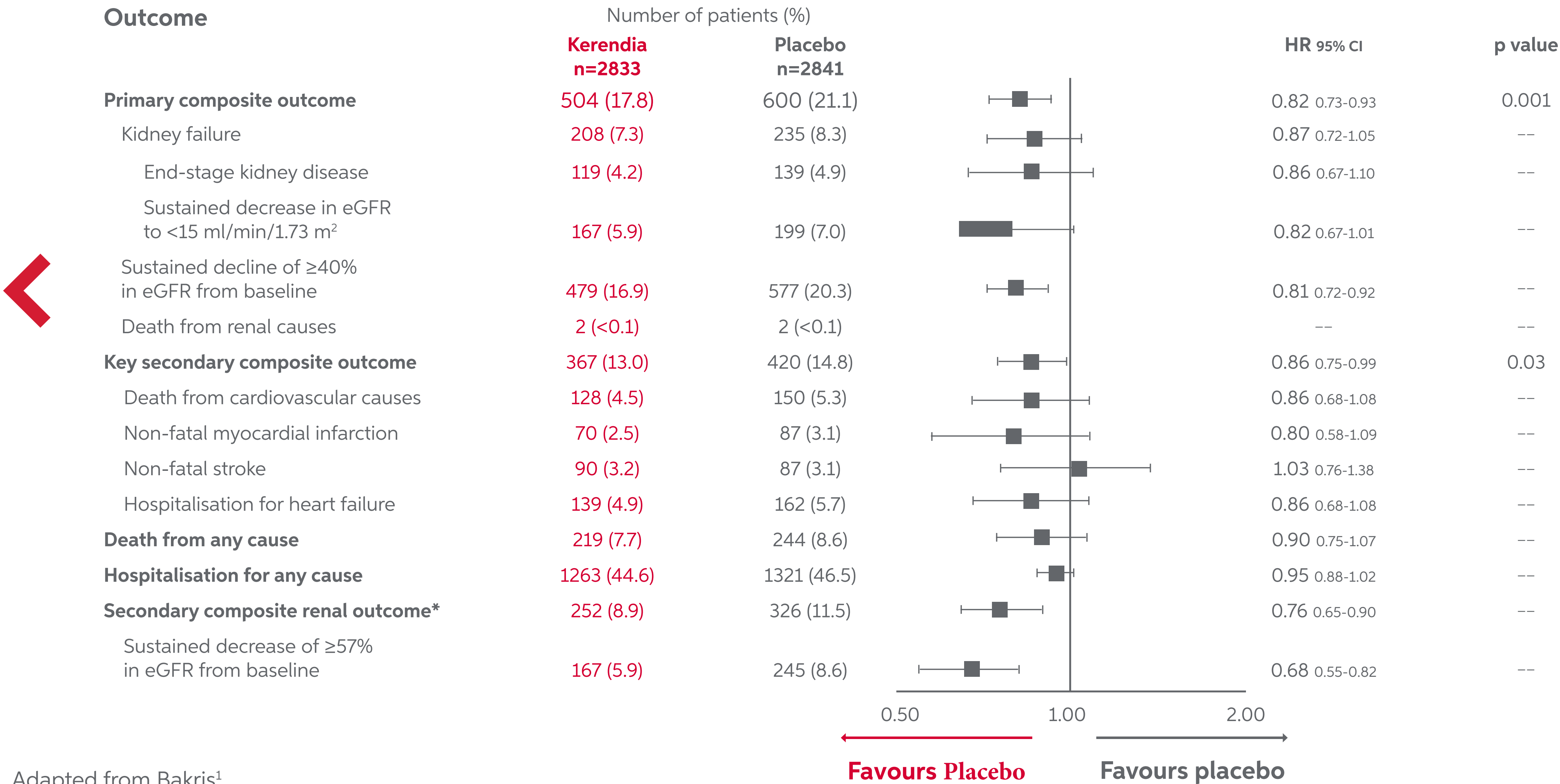


FIDELIO-DKD:

Hierarchical prespecified efficacy outcomes, including the components of the composite outcomes¹

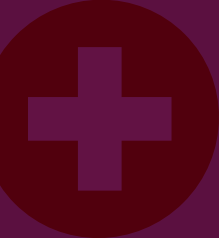


Outcomes were assessed in time-to-event analyses



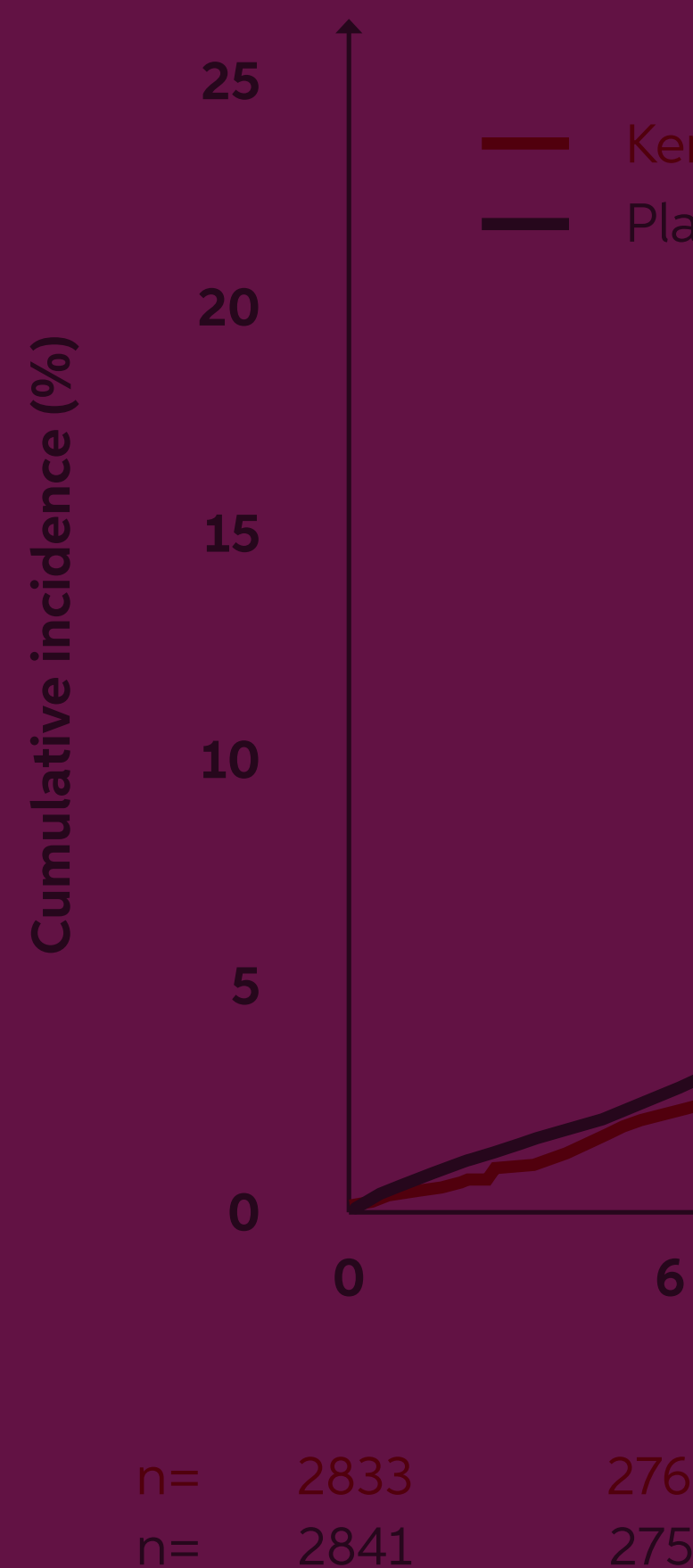
Adapted from Bakris¹

Reduction in risk
of CV events



FIDELIO-DKD:

Secondary composite CV outcome^{1*}



Adapted from Bakris¹

*Secondary composite outcome of kidney failure, a sustained decrease of $\geq 57\%$ in eGFR (equivalent to a doubling of the serum creatinine level), or death from renal causes

ARR: absolute risk reduction
CI: confidence interval
CKD: chronic kidney disease
CV: cardiovascular
CVD: CV disease
DKD: diabetic kidney disease
eGFR: estimated glomerular filtration rate
HbA1c: glycated haemoglobin
HFH: hospitalisation for heart failure
HR: hazard ratio
MI: myocardial infarction
MOA: mechanism of action
MOD: mechanism of disease
RRR: relative risk reduction
T2D: type 2 diabetes
UACR: urine albumin-to-creatinine ratio

References:

1. Bakris GL et al. N Engl J Med 2020; 383(23): 2219-92.
2. Filippatos G et al. Circulation 2021; 143(6): 540-52.

HR: 0.86
95% CI: 0.75-0.99
p=0.03
Median follow-up:
2.6 years

months: 2.4%

secondary
vascular
event:¹

CV causes, or
l, or
roke, or
on for heart failure

Kerendia is not licensed for the treatment of chronic kidney disease (stage 3 and 4 with albuminuria) associated with type 2 diabetes in adult patients.

The risk of CV disease morbidity and mortality, already increased with T2D, is further exacerbated by the presence of CKD²

– analysis of the composite CV outcome was prespecified in FIDELIO-DKD¹

*Secondary composite outcome of death from CV causes, nonfatal MI, nonfatal stroke, or HFH

 **Kerendia**
finerenone



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FIDELIO-DKD:

The incidence of any treatment-emergent adverse event was similar across the Kerendia and placebo groups in patients both with and without a history of CVD^{1*}

Most frequently reported AEs

Safety outcome ^{2*}	% patients (number)	
	Kerendia n=2827	Placebo n=2831
Any AE	87.3% (2468)	87.5% (2478)
AE related to study drug [†]	22.9% (646)	15.9% (449)
AE leading to treatment discontinuation	7.3% (207)	5.9% (168)
Any serious AE	31.9% (902)	34.3% (971)
Serious AE related to study drug [†]	1.7% (48)	1.2% (34)
Serious AE leading to treatment discontinuation	2.7% (75)	2.8% (78)

Adapted from Bakris²

Adverse reactions occurring in >1% of patients on Kerendia treatment are hyperkalaemia, hypotension, hyponatraemia, hyperuricaemia, pruritus and a decreased glomerular filtration rate.³ Please refer to section 4.8 of the Summary of Product Characteristics for further information on undesirable effects³

In patients treated with Kerendia, mean SBP decreased by 2-4mm Hg and mean DBP decreased by 1-2mm Hg at month 1, remaining stable thereafter²

Kerendia has a well tolerated safety profile, with no risks of addiction

^{*}Shown are AEs that occurred during the treatment period, defined as those which started or worsened during Kerendia or placebo intake or up to 3 days after any temporary or permanent interruption

[†]A causal relationship between any AE and administration of Kerendia or placebo was based on the opinion of the reporting investigator

FIDELIO-DKD: shows that Kerendia is Safe and effect



AEs affecting ≥5% of patients in either group^{2*}

AEs	% patients (number)	
	Kerendia n=2827	Placebo n=2831
Hyperkalaemia	15.8% (446)	7.8% (221)
Nasopharyngitis	8.5% (241)	8.8% (250)
Hypertension	7.5% (212)	9.6% (273)
Anaemia	7.4% (209)	6.7% (191)
Peripheral oedema	6.6% (186)	10.7% (304)
Diarrhoea	6.5% (184)	6.7% (189)
Upper respiratory tract infection	6.4% (181)	6.7% (189)
Glomerular filtration rate decreased	6.3% (179)	4.7% (133)
Urinary tract infection	6.3% (179)	6.8% (192)
Back pain	6.2% (175)	6.2% (175)
Hypoglycaemia	5.3% (151)	6.9% (194)
Dizziness	5.2% (146)	5.4% (153)
Arthralgia	5.0% (142)	5.3% (149)
Bronchitis	4.7% (134)	5.3% (151)
Constipation	4.6% (131)	5.8% (163)
Pneumonia	4.5% (128)	6.4% (181)

Investigator-reported renal AEs^{2*}

AEs	% patients (number)	
	Kerendia n=2827	Placebo n=2831
Investigator-reported renal AEs		
Acute kidney injury	4.6% (129)	4.8% (136)
Hospitalisation due to acute kidney injury	1.9% (53)	1.7% (47)
Discontinuation of trial regimen due to acute kidney injury	0.2% (5)	0.2% (7)
Hospitalisation due to acute renal failure	2.5% (70)	2.5% (71)
Discontinuation of trial regimen due to acute renal failure	1.1% (31)	1.3% (36)

The following very common and common adverse reactions are listed in the Great Britain SmPC for Kerendia:

Very common[†]: Hyperkalaemia
Common^{**}: Hyponatraemia, hypotension, pruritis, glomerular filtration rate decrease

*Shown are AEs that occurred during the treatment period, defined as those which started or worsened during Kerendia or placebo intake or up to 3 days after any temporary or permanent interruption

[†]Very common (≥ 1/10)

^{**}Common (≥ 1/100 to < 1/10)

Adapted from Bakris²



FIDELIO-DKD:

The incidence of any treatment-emergent adverse event was similar across the Kerendia and placebo groups in patients both with and without CKD.

Most frequently reported AEs



Safety outcome^{2*}

Any AE

AE related to study drug

AE leading to treatment discontinuation

Any serious AE

Serious AE related to study drug

Serious AE leading to treatment discontinuation

Adapted from Bakris²

The incidence of any treatment-emergent adverse event was similar across the Kerendia and placebo groups in patients both with and without CKD.

Adverse reactions occurred in patients treated with Kerendia included hypotension, hyponatremia, and dizziness. The incidence of hypotension was higher in patients treated with Kerendia than placebo. The incidence of hyponatremia was higher in patients treated with Kerendia than placebo. The incidence of dizziness was higher in patients treated with Kerendia than placebo. Please refer to safety information on undesirable effects³

In patients treated with Kerendia, mean SBP decreased from baseline to month 1 by 3 mmHg²

^{*}Shown are AEs that occurred during the treatment period, defined as those which started or worsened during Kerendia or placebo intake or up to 3 days after any temporary or permanent interruption

[†]A causal relationship between any AE and administration of Kerendia or placebo was based on the opinion of the reporting investigator

- AE: adverse event
- CKD: chronic kidney disease
- CVD: cardiovascular disease
- DBP: diastolic blood pressure
- DKD: diabetic kidney disease
- MOA: mechanism of action
- MOD: mechanism of disease
- SBP: systolic blood pressure
- SmPC: Summary of Product Characteristics
- T2D: type 2 diabetes

References:

1. Filippatos G *et al.* Circulation 2021; 143(6): 540-52.
2. Bakris GL *et al.* N Engl J Med 2020; 383(23): 2219-92.
3. Great Britain Kerendia Summary of Product Characteristics.



Kerendia was associated with no risk of clinically relevant hyperkalaemia^{1,2}

Fidelio-DKD shows that Kerendia is safe

As reflected in the placebo arm, patients in FIDELIO-DKD had a high intrinsic risk of hyperkalaemia because of their advanced CKD, T2D, and treatment with optimised doses of an ACEi or ARB²

The total incidence of treatment-emergent adverse events was similar across the Kerendia and placebo groups. Hyperkalaemia was more common in patients receiving Kerendia vs placebo, with serious events of hyperkalaemia reported in 1.6% of patients receiving Kerendia vs 0.4% receiving placebo¹

Hyperkalaemia-specific events ^{1*}	% patients (number)	
	Kerendia n=2827	Placebo n=2831
Investigator-reported hyperkalaemia [†]	18.3% (516)	9.0% (255)
regimen-related [‡]	11.8% (333)	4.8% (135)
leading to permanent discontinuation of study drug	2.3% (64)	0.9% (25)
leading to hospitalisation	1.4% (40)	0.3% (8)
serious hyperkalaemia	1.6% (44)	0.4% (12)
leading to death	0.0% (0)	0.0% (0)

Treatment-emergent hyperkalaemia thresholds (mmol/l) ¹	% patients (number)	
	Kerendia	Placebo
>5.5	21.7% (597/2785)	9.8% (256/2775)
>6.0	4.5% (126/2802)	1.4% (38/2796)

Adapted from Bakris¹

* Shown are adverse events that occurred during the treatment period, defined as those that started or worsened during Kerendia or placebo intake or up to 3 days after any temporary or permanent interruption

[†]As reported by investigators using the Medical Dictionary for Regulatory Activities (MEDRA) preferred terms “hyperkalaemia” and “blood potassium increased”

[‡]A causal relationship between any AE and administration of Kerendia or placebo was based on the opinion of the reporting investigator

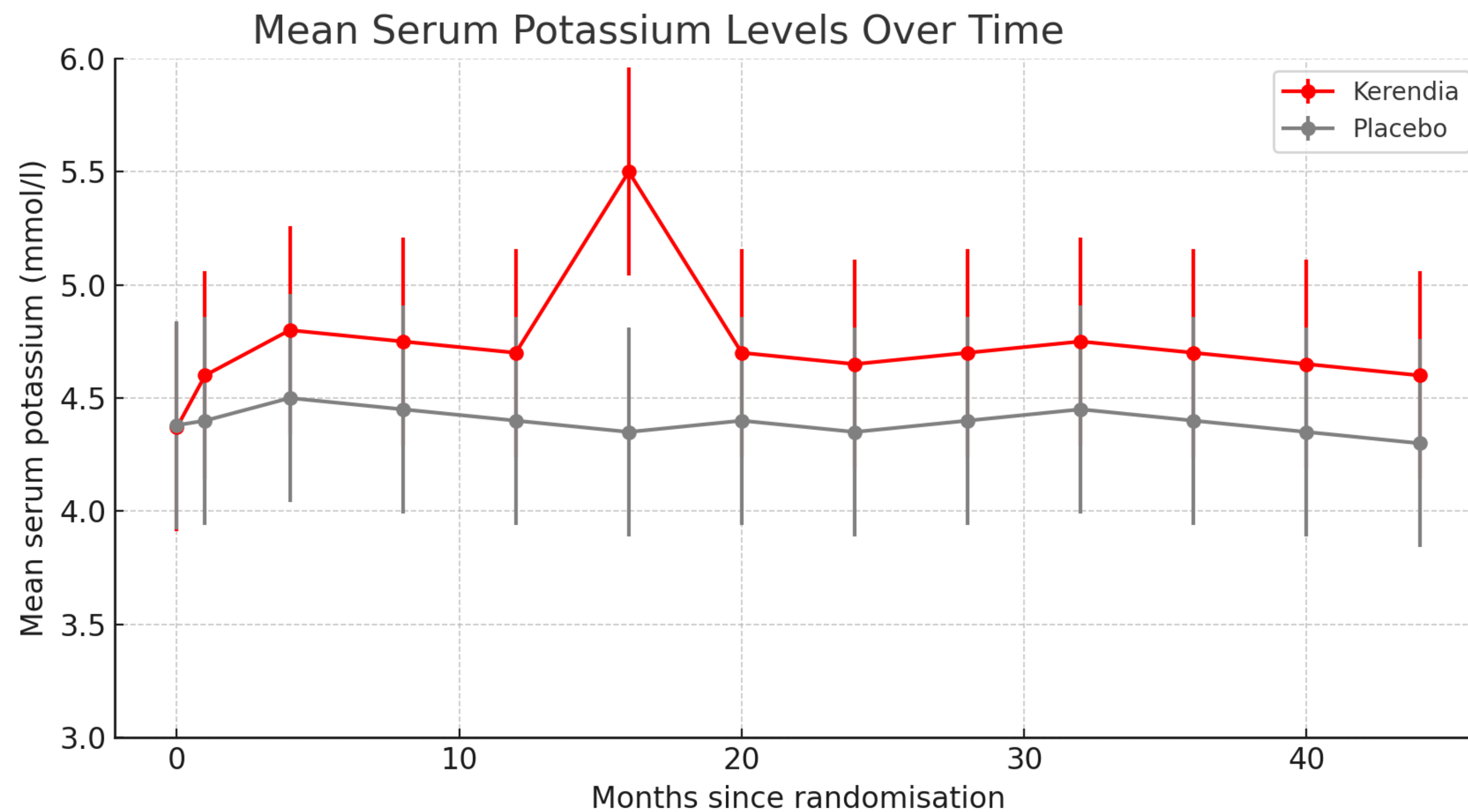
Adapted from Bakris¹

FIDELIO-DKD:

Effects on serum potassium over time in the safety analysis set¹



Maximum mean difference in potassium between groups was 0.23 mmol/l (SD ± 0.5) at month 4, remaining stable thereafter¹



n=	2827	2708	2600	1872	882	344
n=	2831	2709	2596	1865	862	348

Adapted from Bakris¹